

The importance of perioperative chemoimmunotherapy in the treatment of potentially resectable MSI-H gastric adenocarcinoma – a case report

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MSI-H gastroesophageal adenocarcinoma

- 4-24% of gastric and GEJ cancers
- prevalence reflects stage, location, Laurén classification and demographics
- favorable prognostic marker

stage I-II 30%
stage IV 4-12%

antrum 15-30%
body 12-16%
cardia 5-12%
oesophagus 1%

intestinal 10.7%
diffuse 2.9%
mixed 0.9%

females
older age (38% of GC
≥75 years)

GC in 6-13% of Lynch
syndrome patients
MSI-H mostly
sporadic

Case report

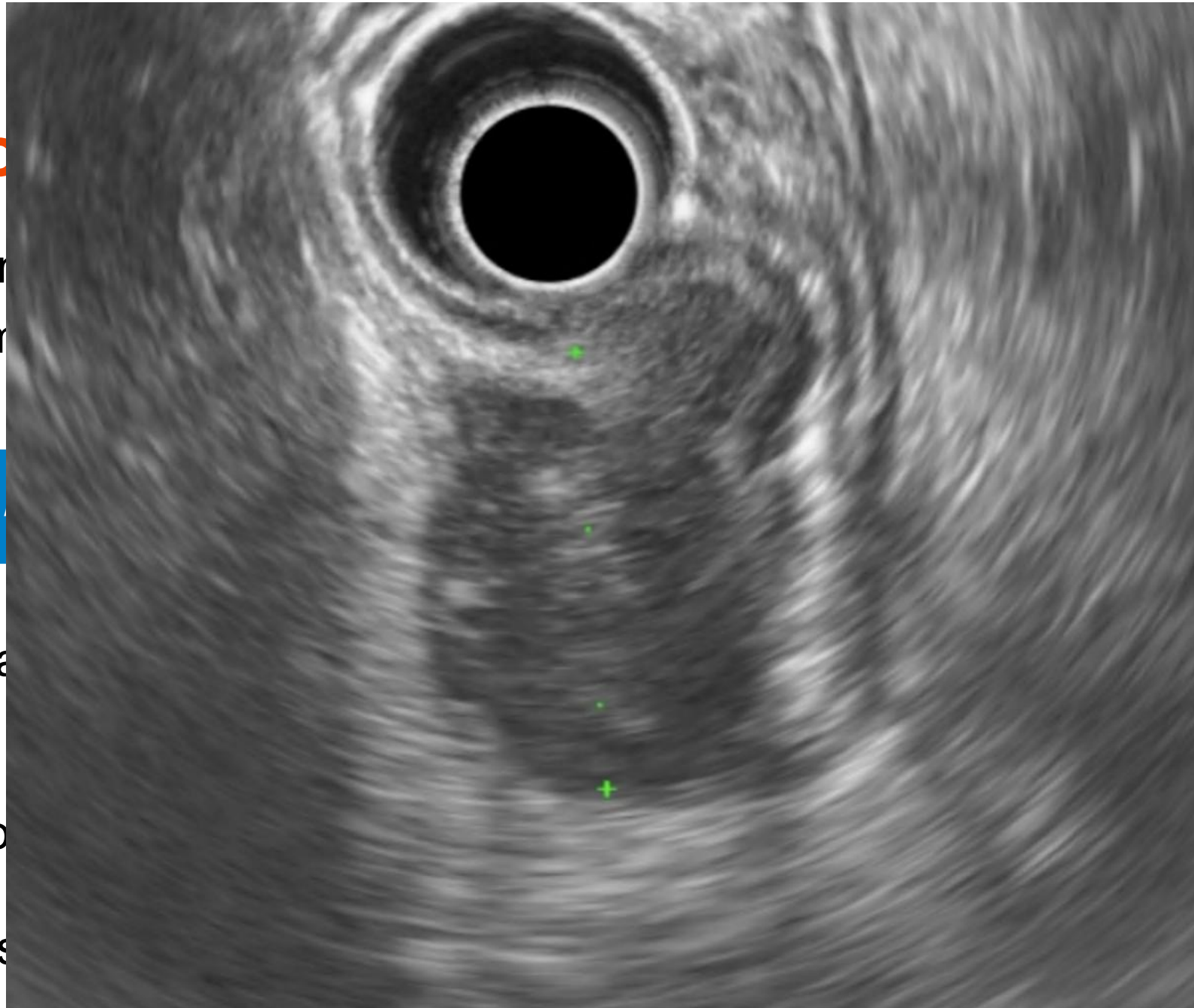
59years old woman
family history: mother

1

GFS finding: gastric
sample)

Gastric endoscopy

CT scan: no dis



Case report - histology

Histology: **poorly cohesive adenocarcinoma with signet ring cells**

Chronic atrophic gastritis, H.pylori colonization G2

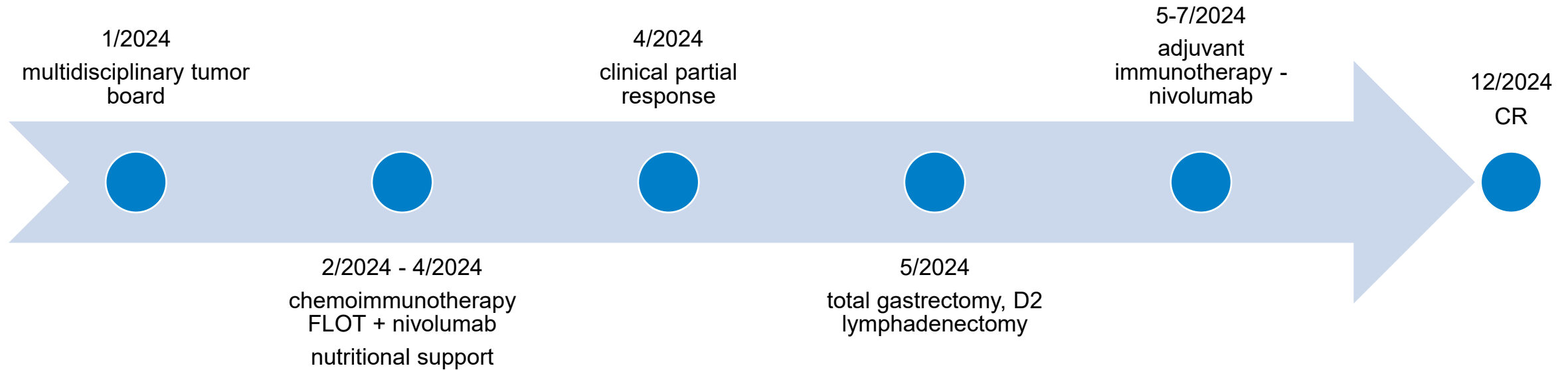
Predictive biomarkers:

HER-2 negative, PD-L1 negative, EBV not examined (not enough material)

MMR deficiency – MSH 2 and 6 mutation

DPYD function score 2

Case report – treatment scheme



Evidence

NEONIPIGA (phase II), INFINITY (phase II), DANTE (phase II/III)

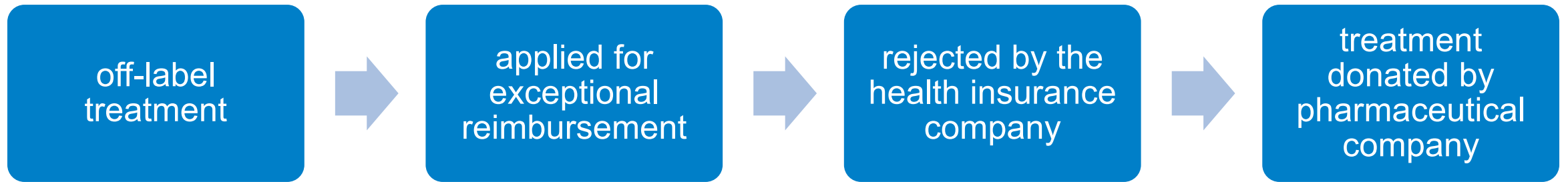
Molecular target	Trial	Publication year	Phase	Treatment	No of patients (MSI-H)	Pathologic complete response	Major pathologic response
dMMR/MSI-H	NEONIPIGA	2022	II	nivolumab + ipiliumumab (anti-PD-1 + anti-CTLA4)	32	59%	73%
dMMR/MSI-H	INFINITY	2023	II	tremelimumab + durvalumab (anti-CTLA4 + anti-PD-1)	17	60%	80%
dMMR/MSI-H or PD-L1 (CPS>1)	DANTE	2024 (interim analysis)	II/III	atezolizumab + FLOT vs. FLOT	25	24% vs. 14%	63% vs. 27%

Pietrantonio, F. et al. (2023). *Journal of Clinical Oncology*, 41(4_suppl), 358–358.

André, T. et al. (2023). *Journal of Clinical Oncology*, 41(2), 255–265.

Lorenzen, S. et al (2024 *Journal of Clinical Oncology*, 42(4), 410–420.

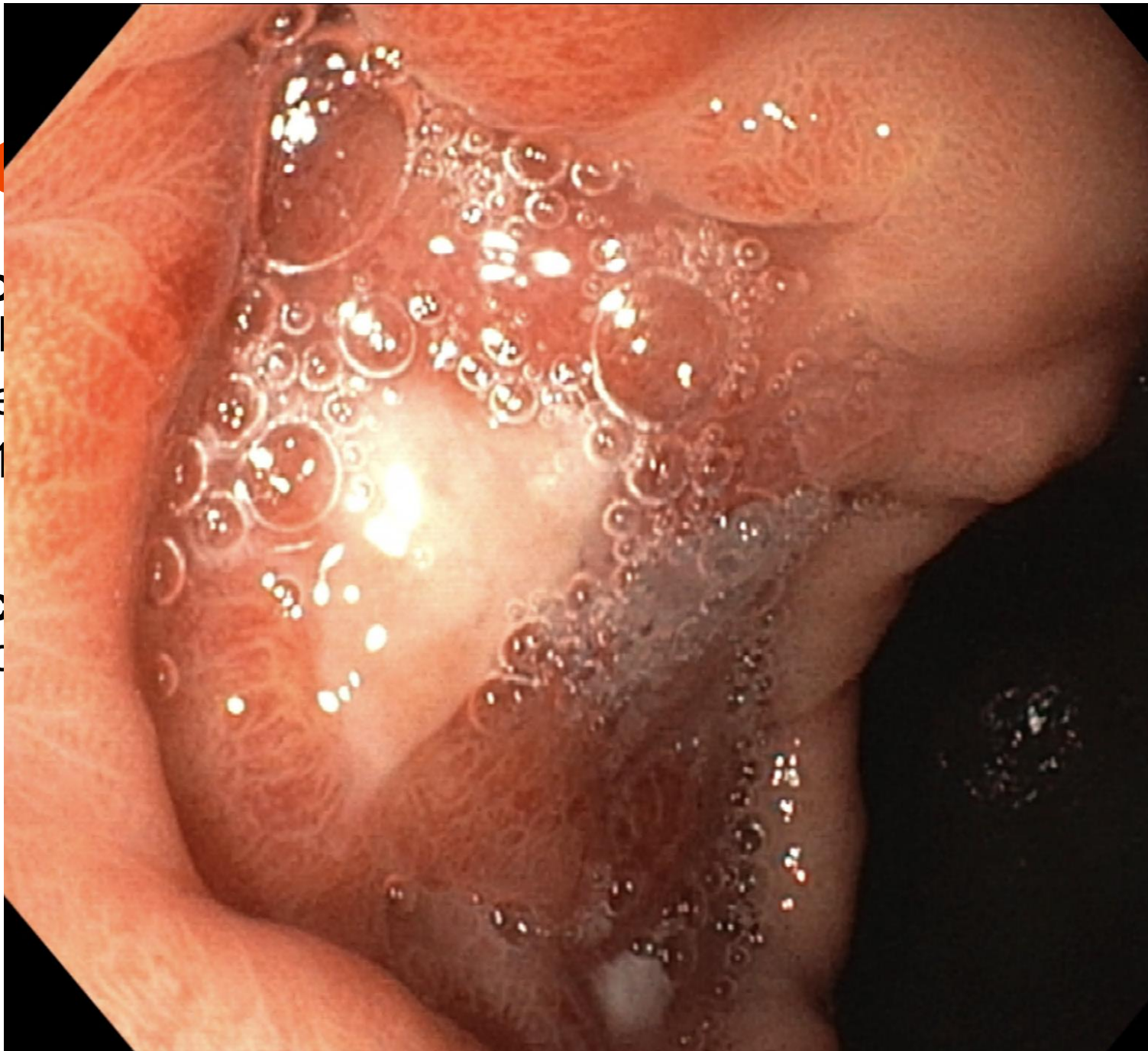
Pitfalls



FLOT + nivolumab (4 cycles)

Treatment

- **partial response**
malignant cells
• no adverse
• anemia G1
- **total gastrectomy**
cholecystectomy
• no surgical
- **ypT1b ypN0**



gression, no

ectomy +

Postoperative treatment

- treatment until 7/2024 - 5 cycles adjuvant nivolumab
 - AE: sensitive lower extremities polyneuropathy, weight loss (12 kg), intermittent diarrhea
- **12/2024 complete remission**
- genetic counseling result 11/2024: **Lynch syndrome**
mutation NM_000179.2 in gene MSH6/c.3261dup/p.Phe1088Leufs*5

Conclusion & discussion

- predictive biomarkers examination importance + genetic examination
- MSI-H EG adenocarcinoma a chemoresistant disease? (FLOT effect? DANTE trial)
- immunotherapy - mono or dual treatment? treatment duration?
- organ preserving strategy? INFINITY trial
- tolerability/adverse effects & medical expenses/reimbursement

Thank you for your attention!

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